

Pipeline Health & Operational Diagnostic Assessment

TO: HHS Program Leadership & Interagency Policy Stakeholders

FROM: Lead Data Analyst & Program Advisor

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TIMEFRAME: January 2023 – December 2025 (720 Reporting Cycles)

1. Executive Summary

This diagnostic evaluation reviews the movement patterns of unaccompanied children (UC) from initial Customs and Border Protection (CBP) custody, through temporary shelter placements managed by the Department of Health and Human Services (HHS), to final sponsor transitions. Over the 3-year analysis window, the operational environment shifted dramatically from a high-density, system-wide capacity squeeze in late 2023 to a highly streamlined, low-census pipeline by the final quarter of 2025.

Crucially, our updated statistical model resolves historical reporting distortions. Rather than utilizing skewed cumulative metrics that frequently exceeded physical limitations due to administrative tracking gaps, we establish a stable operational baseline: the system reliably places **2.80%** of its active daily caseload with sponsors every 24 hours. This translates to an average active placement stay of 35 to 36 days.

2. Key Performance Indicators

Pipeline Segment / Metric	Value	Strategic Significance
Average Daily Apprehensions (CBP)	93.5	Baseline incoming system pressure.
Average Daily Active Backlog (CBP)	171.5	Average pending intake list awaiting shelter placement.
Average Daily Active Bed Census (HHS)	6,061.3	Average daily shelter capacity utilization.
Average Daily Sponsor Placements (HHS)	173.4	Daily operational out-of-system placement speed.
Daily Transfer Efficiency	75.02%	CBP's efficiency in transferring intake to HHS.
Daily Discharge Index	2.86%	Active caseload percentage placed with sponsors daily.
Pipeline Throughput Rate	2.80%	Real-time capacity clearance rate (Exits / total active load).

3. Temporal & Operational Milestones

The operational lifecycle of the pipeline over the 720-day period was shaped by two polar-opposite periods of system stress:

- **System Peak Capacity Crisis (December 20, 2023):** Active shelter occupancy reached an all-time high of **11,516 children**. During this period, physical intake significantly outpaced sponsor placement pipelines, driving up average lengths of stay, straining bed availability, and ballooning short-term operational costs.
- **System Low-Water Mark (August 21, 2025):** The active census dropped to a historical floor of **1,972 children**. This contraction represents a combined effect of reduced border apprehensions and matured, fast-tracked local sponsor discharge protocols.

4. Key Operational Findings & Data Audit

During our deep-dive audit of the source dataset, two key administrative dynamics were surfaced:

- **Administrative Reporting Gaps:** Total cumulative exits (\$124,853\$ children) exceeded direct CBP transfers (\$92,641\$ children). This statistical leak occurs because children are frequently admitted to HHS custody from non-CBP sources (e.g., ICE referrals or direct local actions) or are subject to retroactive inventory reconciliations. Reporting on simple cumulative sums creates highly distorted 'efficiencies' (e.g., \$134.77\%\$ throughput), making our transition to active daily clearance modeling a necessity for true operational transparency.
- **Co-dependency of Placement Velocity:** Correlation testing indicates that HHS sponsor placements scale linearly with active bed census. This indicates that exit pathways are heavily throttle-controlled by shelter capacities—when caseloads peak, placement infrastructure matches the scale, meaning throughput is

dynamic rather than fixed.

5. Policy & Operational Recommendations

- **Transition to Active Volume Modeling:** Cease the use of cumulative sums for long-term pipeline planning. Regional directors should be evaluated using the **Daily Discharge Index (Target: 3.2%)** to ensure consistent placement processing speed regardless of absolute numbers.
- **Establish an Elastic Bed Reserve:** With historical demand swinging between \$1,972\$ and \$11,516\$ children, HHS should formalize an elastic capacity strategy. Maintaining a ready-state buffer of **4,000 warm-status beds** allows the network to scale within 48 hours to sudden border surges without maintaining high daily overhead costs during low-intake seasons.
- **Automate Low-Risk Placement Tracks:** Sponsor vetting processes should be tiered based on relation. Streamlining and automating background clearances for immediate parents and legal guardians (who represent the lowest risk) will accelerate daily clearance ratios, reducing average length of stay below the 30-day mark.